



1. Which of the following represents the surface anatomy of the apex of the heart?

* 1 point

- ☒ A. Lies opposite the left 5th intercostal space 9 cm from the middle line
- ☐ B. Lies opposite the right 5th intercostal space 9 cm from the middle line.
- ☐ C. Lies opposite the left 6th intercostal space.
- ☐ D. Lies opposite the 4th thoracic vertebrae.

2. The most posterior structure of heart is.

* 1 point

- ☒ A- left atrium
- ☐ B- Right atrium
- ☐ C- Left ventricle
- ☐ D- Right atrial appendage

3. Which valves are commonly affected by rheumatic fever?

* 1 point

- ☐ A- Tricuspid, mitral
- ☐ B- Mitral, Aortic
- ☒ C- Tricuspid, pulmonary
- ☐ D- Aortic , pulmonary.

4. Prophylactic antibiotic is indicated before dental extraction in the following:

* 1 point

- ☐ A- Coronary artery bypass.
- ☒ B- Prosthetic aortic valve

4. Prophylactic antibiotic is indicated before dental extraction in the following:

* 1 point

- ☐ A- Coronary artery bypass.
- ☒ B- Prosthetic aortic valve
- ☐ C- Chronic kidney disease
- ☐ D- Liver cell failure

5. Fever, Clubbing and Osler's nodes occur in:

* 1 point

- ☐ A- Leptospirosis
- ☐ B- Typhoid fever
- ☒ C- Infective endocarditis
- ☐ D- Rheumatic fever.

6. In a patient suffering from congenital cyanotic heart diseases who has to go for dental procedure, when are prophylactic drugs given:

* 1 point

- ☒ A- One Hour before operation.
- ☐ B- 7 days before operation

6. In a patient suffering from congenital cyanotic heart disease who has to go for dental procedure, when are prophylactic drugs given:

* 1 point

- ☒ A- One Hour before operation.
- ☐ B- 7 days before operation
- ☐ C- 3 Hours before operation
- ☐ D- 1 day before operation

7. The causative organism of rheumatic heart disease associated with rheumatic fever:

* 1 point

- ☐ A- Beta hemolytic staphylococci
- ☒ B- Beta hemolytic streptococci
- ☐ C- Alpha hemolytic streptococci
- ☐ D- Gamma hemolytic streptococci.

8. Characteristic pathologic feature of rheumatic fever is/are:

* 1 point

- ☐ A- Fever
- ☒ B- Joint pain
- ☐ C- Aschoff nodules
- ☐ D- Headach

9. Physical sign of severe aortic stenosis is:

* 1 point

- ☐ A- A diastolic rumble murmur.
- ☐ B- Delayed carotid upstroke.
- ☐ C- An opening snap
- ☒ D- A late peaking holosystolic murmur.

10. De-Musset sign in Aortic regurgitation is

* 1 point

- ☒ A- Bobbing of head
- ☐ B- Nail pulsation
- ☐ C- Neck pulsation
- ☐ D- Pulsatile hepatomegaly.

11. In which of the following cases apical diastolic murmur occur?

* 1 point

- ☐ A- Ventricular septal defect.
- ☐ B- Mitral stenosis
- ☐ C- Aortic stenosis
- ☒ D- Mitral regurgitation.

12. Hemoptysis is a characteristic feature of:

* 1 point

- ☒ A- Mitral stenosis
- ☐ B- Aortic stenosis
- ☐ C- Mitral regurgitation
- ☐ D- Aortic regurgitation.

13. A 52-year-old woman arrived to the ER (emergency room), C/O of repeated syncopal attacks and history of exertional chest pain, On history she reported history of chronic cardiac problem of uncertain nature, on examining her pulse rate was 84 bpm, of small volume. Which is the most likely professional diagnosis. * 1 point

- ☐ A. Aortic regurgitation
- ☐ B. Mitral regurgitation
- ☒ C. Aortic stenosis
- ☐ D. Tricuspid regurgitation

14. Wide fixed split * 1 point

- ☒ A- ASD
- ☐ B- MS
- ☐ C- Pulmonary embolism,
- ☐ D- Constrictive pericarditis.

15. Which of the following describes Ventricular septal defect murmur? * 1 point

- ☒ A. Holosystolic murmur
- ☐ B. Ejection systolic murmur
- ☐ C. Systolic-diastolic murmur
- ☐ D. None of the above.

16. An infant with a marked cyanotic congenital heart defect with decreased pulmonary vascularity should be treated with: * 1 point

16. An infant with a marked cyanotic congenital heart defect with decreased pulmonary vascularity should be treated with:

* 1 point

- ☐ A. Digoxin
- ☐ B. Prostaglandin E1
- ☒ C. Indomethacin
- ☐ D. Epinephrine.

17. What is the function of the AV node?

* 1 point

- ☐ A. Serve as back-up pacemaker
- ☐ B. Limit fibrillation entry into ventricles
- ☐ C. Delay action potential to allow ventricular filling.
- ☒ D. All of the above.

18. Which type of cardiomyopathy in which cardiac muscle becomes stiff and restricts blood from filling ventricles, usually because of amyloidosis, radiation, or myocardial fibrosis after open-heart surgery?

* 1 point

- ☐ A- Hypertrophic cardiomyopathy.
- ☐ B- Dilated cardiomyopathy
- ☒ C- Restrictive cardiomyopathy
- ☐ D- Viral cardiomyopathy

19. When fluid builds up in the lungs due to ineffective pumping of blood by the heart as a result of left-sided heart failure, AMI, worsening of heart failure or volume overload.

* 1 point

- ☐ A. Hypovolemic shock

19. When fluid builds up in the lungs due to ineffective pumping of blood by the heart as a result of left-sided heart failure, AMI, worsening of heart failure or volume overload.

★ 1 point

- ☐ A- Hypovolemic shock
- ☐ B- Pericarditis
- ☒ C- Pulmonary edema
- ☐ D- Myocarditis.

Which of the following conditions is most closely associated with weight gain, nausea, and a decrease in urine output?

★ 1 point

- ☐ A. Angina pectoris
- ☐ B. Cardiomyopathy
- ☐ C. Left-sided heart failure
- ☒ D. Right-sided heart failure

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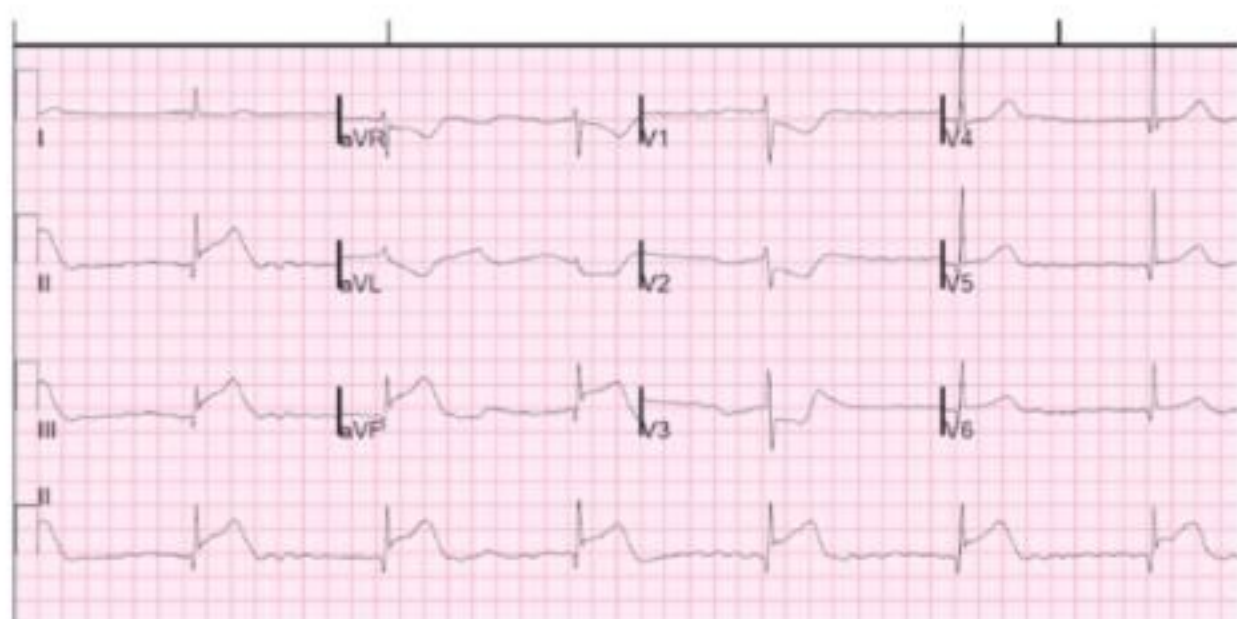
✓ 1- A 55-year-old man has just arrived in ER complaining of 20 minutes of central crushing chest pain. Which feature is most indicative of myocardial infarction at this moment in time? *1/1

- ☐ A) ST depression
- ☒ B) ST elevation ✓
- ☐ C) Q waves
- ☐ D) Sinus tachycardia

✓ 1- A 60-year-old man presents with a 3-day history of increasingly severe chest pain. The patient describes the pain as a sharp, tearing pain starting in the center of his chest and radiating straight through to his back between his shoulder blades. The patient looks in pain but there is no pallor, heart rate is 95, respiratory rate is 20, temperature 37°C and blood pressure is 155/95 mmHg in the right arm. The most likely diagnosis is: *1/1

- ☐ A) Myocardial infarction
- ☐ B) Myocardial ischemia
- ☒ C) Aortic dissection ✓
- ☐ D) Pulmonary embolism

✓ 4- What is the spot diagnosis of *1/1
the following ECG?



☐ A. Acute anterior myocardial infarction

☒ B. Acute inferior myocardial infarction. ✓

☐ C. Unstable angina.

☐ D. NSTEMI

☐ Other: _____

✗ 4- High blood pressure is the *0/1
main cause of which of these?

☒ A) Cancer ✗

☐ B) Stroke

☐ C) Diabetes

☐ D) Liver cell failure.

Correct answer

☒ B) Stroke

✓ 4- A 56-year-old man presents to your clinic with symptoms of exertional chest tightness which is relieved by rest. You request an ECG which reveals that the patient has first degree heart block. Which of the following ECG abnormalities is typically seen in first degree heart block? *1/1

- ☐ A. PR interval >120 ms
- ☐ B. PR interval >300 ms
- ☐ C. PR interval <200 ms
- ☒ D. PR interval >200 ms ✓

✓ 4- This patient came to ER C/O from palpitation and dizziness. What is the rhythm in the following ECG: *1/1



- ☒ A. sustained monomorphic ventricular tachycardia ✓
- ☐ B. Intermittent right bundle branch block
- ☐ C. Artifacts
- ☐ D. Paroxysmal atrial fibrillation with aberration.

✓ 4- Features that suggest a ventricular rather than supraventricular tachycardia include: *1/1

- ☒ A. atrioventricular dissociation ✓
- ☐ B) Termination of arrhythmia with carotid sinus pressure.
- ☐ C) Variable intensity of first heart sound
- ☐ D) QRS complex <140ms in duration on ECG.

✓ **4- Features that suggest a ventricular rather than supraventricular tachycardia include:**

*1/1

- ☒ **atrioventricular dissociation** ✓
- ☐ B) Termination of arrhythmia with carotid sinus pressure.
- ☐ C) Variable intensity of first heart sound
- ☐ D) QRS complex <140ms in duration on ECG.

✓ **Which investigation is the most appropriate in assessment of pericardial effusion.**

*1/1

- ☐ A) ECG.
- ☐ B) Chest x-ray.
- ☒ **C) Transthoracic echo** ✓
- ☐ D) Cardiac enzymes.

✓

*1/1

4- Typical features of acute pericarditis

- ☐ A) Chest pain is identical of myocardial infarction.
- ☒ **B) Friction rub best heard over lower sternal border at mid expiration.** ✓
- ☐ C) ECG changes only seen in anterior chest leads
- ☐ D) Elevation of serum creatin kinase.

✓ **4- Classic exercise ECG finding in patient with ischemic heart disease**

*1/1

- ☒ **A) ST depression** ✓
- ☐ B) ST elevation
- ☐ C) P-R depression
- ☐ D) P-R elevation.

✓ **4- Continuous and intermittent recorders are types of:** *1/1

- ☒ A) Ambulatory ECG monitoring ✓
- ☐ B) Ambulatory BP monitoring
- ☐ C) Surface ECG
- ☐ D) Stress ECG.

✓ **The risk of dissecting aortic aneurysm is increased in** *1/1

- ☒ A) Marfan Syndrome ✓
- ☐ B) Rheumatic AS
- ☐ C) Coarctation of the aorta.
- ☐ D) Calcific aortic stenosis.

✓ **4- The angle of trachea is increased in which chamber enlargement of the heart.** *1/1

- ☒ A) Left atrium. ✓
- ☐ B) Right atrium.
- ☐ C) Left ventricle.
- ☐ D) Right ventricle.

✓ **Echocardiography is most useful for detecting:** *1/1

- ☒ A) Ventricular dilatation ✓
- ☐ B) Midline shift.
- ☐ C) cerebral insufficiency
- ☐ D) epilepsy

✓ **17- The best tool for evaluation of posterior cardiac structures** *1/1

- ☐ A) Transthoracic Echocardiography
- ☐ B) ECG
- ☐ C) Chest X-ray
- ☒ D) Transesophageal echocardiography. ✓

✓ **18- Which of the following is the most common primary tumor of heart in adults?** *1/1

- ☒ A. Myxoma ✓
- ☐ B. Rhabdomyoma
- ☐ C. Fibroma
- ☐ D. D-Hemangioma

✓ 19- Beck triad (the 3 Ds, *1/1
distended neck veins, distant heart
sound and decreased blood
pressure is present in which of the
following?

- ☐ A) Congestive heart failure (CHF)
- ☐ B) Cardiac shock
- ☐ C) Cardiomyopathy
- ☒ D) Cardiac Tamponade ✓

✓ 20 In which type of angina does *1/1
pain occur at rest; of new onset; of
increasing intensity, force, or
duration; not relived by rest; and is
slow to subside in response to
nitroglycerin?

- ☐ A) Prinz metal's or vasospastic angina
- ☐ B) Stable angina
- ☒ C) Unstable angina ✓
- ☐ D) Musculoskeletal pain.

✗ 1- Masked hypertension is defined as:

*0/1

- ☐ A) Normal office and home BP
- ☐ B) Normal office BP and elevated home BP
- ☒ C) Elevated office BP and normal home BP ✗
- ☐ D) Elevated office and home BP

Correct answer

- ☒ B) Normal office BP and elevated home BP

✗ 4- 58-year-old man with history *0/1
of old anterior myocardial
infarction. Echocardiography
showed severely reduced left
ventricular systolic function.
Which of the following is the right
answer:

☐ A. dilated left ventricle with ejection
fraction 60%

☒ B. normal sized left ventricle with
ejection fraction 55%

✗

☐ C. hypertrophied left ventricle with
ejection fraction 50%

☐ D. Dilated left ventricle with ejection
fraction 30%

☐ خيار 5

Correct answer

☒ D. Dilated left ventricle with ejection
fraction 30%



1/1*

Which of the following .1
statements is true about cardiac
?edema



A. It occurs only in the lower limbs ☐

✓ B. It is due to congestive heart failure ☒

C. It is non pitting edema ☐

D. Edema fluid is composed of inflammatory exudates ☐

1/1*

Rheumatic Fever usually .2
:follows an infection with



✓ A. Streptococcus group A ☒

B. Staphylococcus aureus ☐

C. Streptococcus viridians ☐

D. H.influenza ☐

1/1*

Which valves are commonly .3
:affected by rheumatic fever



1/1* Which valves are commonly .3 ✓
:affected by rheumatic fever

A. Tricuspid , mitral ☐



B. Mitral , Aortic ☒

C. Tricuspid , pulmonary ☐

D. Aortic , pulmonary ☐

1/1* The mitral stenosis murmur .4 ✓
is



A. Mid diastolic rumbling murmur ☒

B. Pan systolic murmur ☐

C. Ejection systolic murmur ☐

D. Increased by inspiration ☐

1/1* Which murmur commonly .5 ✓
?radiates to the axilla

A. Aortic stenosis. ☐



B. Mitral regurgitation ☒

C. Aortic regurgitation. ☐



1/1* What is the correct pathway .6 ✓
for the heart's conducting
system

- A. SA Node->AV Node->Atrial Muscle->AV Bundle->Purkinje Fibres->Ventricular Muscle ☐
- B. SA Node->AV Node->AV Bundle->Purkinje Fibres->Atrial Muscle->Ventricular Muscle ☐
- C. SA Node->Atrial Muscle->AV Node->AV Bundle->Ventricular Muscle->Purkinje Fibres ☐
- ✓ D. SA Node->Atrial Muscle->AV Node->AV Bundle->Purkinje Fibres->Ventricular Muscle ☒

1/1* What is the resting potential .7 ✓
of an SA Nodal cell



A. -60 mV ☒

B. -70 mV ☐

C. -90 mV ☐

D. +50 mV ☐

1/1* What is the function of the .8 ✓
?AV node

- A. Serve as back-up pacemaker ☐
- B. Limit fibrillation entry into ventricles ☐
- C. Delay action potential to allow ventricular filling ☐
- ✓ D. All of the above ☒

1/1 * Select the true statement .9 ✓

- ✓ A. Mitral and tricuspid stenosis is part of diastolic murmur ☒
- B. Mitral and tricuspid regurgitation is part of diastolic murmur ☐
- C. Aortic and pulmonic stenosis is part of diastolic murmur ☐
- D. Aortic and pulmonic regurgitation is part of systolic murmur ☐

1/1* the most frequent symptom .10 ✓
experienced by patients with
dilated cardiomyopathy is



1/1* the most frequent symptom .10 ✓
experienced by patients with
dilated cardiomyopathy is

A. Hypertension ☐

B. Increased cardiac output ☐



C. Dyspnea ☒

D. hemoptysis ☐

1/1* If your patient has right .11 ✓
heart failure, what classic signs
and symptoms would you expect
?to see

A. Atrial fibrillation ☐

B. Hymoptysis ☐

C. Orthopnea ☐



D. Lower extremity edema ☒

1/1* A cardiomyopathy is a .12 ✓
disease that diffusely affects
the____, resulting in enlargement
and/or ventricular dysfunction



1/1*

A cardiomyopathy is a .12 ✓
disease that diffusely affects
the____, resulting in enlargement
and/or ventricular dysfunction



A. Myocardium ☒

B. Heart valves ☐

C. Epicardium ☐

D. Endocardium ☐

1/1*

One of the following is not a .13 ✓
physical symptom in patients
with congestive dilated
:cardiomyopathy

A. Sinus tachycardia ☐

B. Edema ☐

C. Fatigue ☐



D. Systemic hypertension ☒

1/1*

What is a primary cause of .14 ✓
?myocarditis



1/1* What is a primary cause of .14 ✓
?myocarditis

A. Myocardial infarction ☐

B. Coronary heart disease ☐

✓ C. Bacterial or viral infection ☒

D. Stroke ☐

1/1* Mitral valve according to it is .15 ✓
:structure also is called

✓ Bicuspid ☒

Tricuspid ☐

C. Unicuspid ☐

Quadricuspid ☐

1/1* Following a car accident, a .16 ✓
52-year old woman complains of
pain in the left abdomen and
back. After arrival of the EMS
(emergency medical service)

1/1* Following a car accident, a 52-year old woman complains of pain in the left abdomen and back. After arrival of the EMS (emergency medical service) team, her pulse rate is 84 bpm, but of small volume. She states that she has some cardiac condition but is uncertain of its nature. Which is the most likely cause of the small pulse volume? ✓

A. Aortic regurgitation ☐

B. Mitral regurgitation ☐

✓ C. Aortic stenosis ☒

D. Tricuspid regurgitation ☐

1/1* A 12-year-old boy develops cardiac symptoms that were attributed to rheumatic fever. At the age of 34 he is admitted to the hospital and examination reveals ejection systolic murmur at the base. Which of the ✓



1/1* A 12-year-old boy develops cardiac symptoms that were attributed to rheumatic fever. At the age of 34 he is admitted to the hospital and examination reveals ejection systolic murmur at the base. Which of the following investigations is the method of choice to make diagnosis and specify valvular heart disease ✓



A. Echocardiography ☒

B. ECG ☐

C. Holter monitoring ☐

D. Cardiac catheterization ☐

0/1* 45 year old female patient with history of mitral stenosis. Which of the following conditions can not be a result of her current valvular lesion ✗



A. Atrial fibrillation ☒

B. LV dilatation ☐

C. Pulmonary hypertension ☐

D. Right ventricular dilation ☐



0/1*

year old female patient 45 .18 ✕

with history of mitral stenosis.

Which of the following conditions can not be a result of :her current valvuar lesion



A. Atrial fibrillation ☒

B. LV dilatation ☐

C. Pulmonary hypertension ☐

D. Right ventricular dilation ☐

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B. LV dilatation ☒

1/1*

year old man and 22 .19 ✓

presented to emergency department by collapse after experiencing rapid racing heart beats. He had past history of mitral stenosis. Which arrhythmia develops most often ?in such patient

A. Supraventricular tachycardia ☐

B. Atrioventricular block ☐



C. Atrial fibrillation ☒



1/1*

A 33-year-old man with no past medical history is admitted for flu-like symptoms, shortness of breath, fatigue, and leg edema. He is managed conservatively. Which of the following diagnosis is suspected



A. Viral myocarditis ☒

B. Acute rheumatic fever ☐

C. Systemic lupus ☐

D. Myocardial infarction ☐

1/1*

Which diagnostic tool is necessary to confirm a diagnosis of COPD



A. ECG ☐

B. Chest radiography ☐

C. Pulse oximetry ☐



D. Spirometry. ☒



1/1

* Chronic bronchitis is .22 ✓



A. A result of smoking for years ☒

B. An episode of coughing lasting a month ☐

C. Often found in children ☐

D. A result of a viral illness ☐

1/1*

In an advanced stage of .23 ✓
COPD, what treatment might be
prescribed

A. Chemotherapy ☐

B. Bronchodilators ☐



C. Pulmonary rehabilitation ☒

D. Antibiotics ☐

1/1*

All of the following can be .24 ✓
used to diagnose atypical CAP
EXCEPT

A. ESR and C-reactive protein. ☐



1/1* All of the following can be .24 ✓
used to diagnose atypical CAP
?EXCEPT

A. ESR and C-reactive protein. ☐

B. X-rays. ☐



C. Gram stain. ☒

d. pcr ☐

1/1* Which of the following .25 ✓
regarding pulmonary
?hypertension (PH) is FALSE

A. PH has many underlying etiologies ☐

✓ B. The disease affects the left side
of the heart ☒

C. The hemodynamic definition of PH
is a mean pulmonary arterial pressure
(PAP) greater than or equal to 25
mmHg ☐

D. Pulmonary arterial hypertension
(PAH) is a subset of PH ☐



1/1*

Which of the following statements about angina is not true

.1



A. Typically lasts for 1 to 5 minutes ☐

B. Pain can radiate to both arms ☐

✓ C. Pain localized below umbilicus or above mandible ☒

D. Pain character can be squeezing, burning, or heaviness in the chest. ☐

1/1*

Angina decubitus" refers to angina pain occurring when the patient is in which of the following positions

.2



A. Squatting ☐

B. Sitting with legs hanging ☐

✓ C. Lying down ☒

D. Standing ☐



1/1* Which of the following drugs 2 ✓

1/1* Which of the following drugs .3 ✓
is not useful in the management
?of angina pectoris

A. Beta blockers ☐

B. Nitrates ☐

C. Antiplatelet ☐



D. Paracetamol. ☒

1/1* What is the most common .4 ✓
congenital heart defect with left
to right shunt causing congestive
heart failure in the pediatric age
?group

A. Atrial septal defect ☐



B. Ventricular septal defect ☒

C. Atrioventricular canal ☐

D. Patent ductus arteriosus ☐

0/1* A 2 year old infant is noted .5 ✗
to have mild cyanosis when
assuming squatting position



0/1* A 2 year old infant is noted .5 ✗
to have mild cyanosis when
assuming squatting position
during long walking. The most
likely underlying lesion is

- A. Tetralogy of Fallot ☐
- B. Ventricular septal defect ☐
- C. Transposition of the great vessels ☐
- ✗ D. Co-arctation of the aorta ☒

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A. Tetralogy of Fallot ☒

1/1* An infant with a marked .6 ✓
cyanotic congenital heart defect
with decreased pulmonary
vascularity should be treated
:with

- A. Digoxin ☐
- ✓ B. Prostaglandin E1 ☒
- C. Indomethacin ☐
- D. Epinephrine ☐



1/1* Cyanosis is produced by the .7 ✓
presence of deoxygenated
:hemoglobin of at least

A. 1-2 gm/dL ☐



B. 3-5 gm/dL ☒

C. 6-8 gm/dL ☐

D. 9-10 gm/dL ☐

0/1* Which of the following is not .8 ✗
a line of treatment for the
cyanotic spell of tetralogy of
:Fallot

A. Oxygen ☐



B. Knee chest position ☒

C. Propranolol ☐

D. Digoxin ☐

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D. Digoxin ☒

1/1* Which of the following .9 ✓



1/1*

Which of the following .9 ✓
represents the surface anatomy
?of the apex of the heart

✓ A. Lies opposite the left 5th
intercostal space 9 cm from the
middle line ☒

B. Lies opposite the right 5th
intercostal space 9 cm from the middle
line ☐

C. Lies opposite the left 6th intercostal
space ☐

D. Lies opposite to 4th thoracic
vertebrae ☐

1/1*

The left subclavian artery .10 ✓
arises from

A. Innominate artery ☐

B. Descending aorta ☐

✓ C. The arch of the aorta ☒

D. Ascending aorta ☐

1/1

*:The left coronary artery .11 ✓



1/1 * :The left coronary artery .11 ✓

A. Arise from the anterior surface of the aorta ☐

B. Supplies the posterior interventricular branch ☐

C. Supplies the diaphragmatic part of right ventricle ☐

✓ D. Pass between the pulmonary trunk and left auricle ☒

1/1* Pericardial cavity normally .12 ✓
?contains how much fluid

A. 5- 10 ml of fluid ☐

B. 10-20 ml of fluid ☐

✓ C. 15-50 ml of fluid ☒

D. 50-100 ml of fluid ☐

1/1* Pericardial friction rub is .13 ✓
?best heard at

✓ A. Left lower sternal border ☒



1/1*

Pericardial friction rub is .13 ✓
?best heard at



A. Left lower sternal border ☒

B. Apex ☐

C. Right sternal border ☐

D. Infraclavicular area ☐

1/1*

Which of the following is the .14 ✓
most common primary tumor of
?heart in adults



A. Myxoma ☒

B. Rhabdomyoma ☐

C. Fibroma ☐

D. Hemangioma ☐

1/1*

Characteristic endocardial .15 ✓
lesions of SLE (Libman & Sacks)
?are most often located at

A. Atria ☐



1/1* Characteristic endocardial .15 ✓
lesions of SLE (Libman & Sacks)
?are most often located at

A. Atria ☐

B. Left ventricular outflow tract ☐

✓ C. Ventricular surface of the mitral valve ☒

D. Papillary muscle ☐

1/1* Which of the following about .16 ✓
?Third-degree AV block is false

A. No atrial impulse propagates to the ventricles ☐

B. Congenital complete heart block is localized to the AV node ☐

C. In His bundle block, QRS duration is prolonged ☐

✓ D. In AV nodal block, QRS duration is prolonged ☒

1/1* Escape pacemaker .17 ✓



1/1*

Escape pacemaker .17 ✓
following AV nodal block is
?usually in



A. His bundle ☒

B. Bundle branches ☐

C. Purkinje fibers ☐

D. Ventricular myocardium ☐

1/1*

Which of the following .18 ✓
statements about escape
pacemaker in distal His-Purkinje
?system is false

A. Has a rate of 25 to 45 beats per
minute ☐

B. Wide QRS complex of prolonged
duration ☐

C. Is unstable ☐

✓ D. Is the escape pacemaker
followed by AV nodal block ☒

1/1*

Echocardiography can be .19 ✓
used for



1/1*

Echocardiography can be .19
used for



. Remove embolus of the heart



B. Correctly locate the blockages in the
arteries and veins



✓ C. Assess the baby's heart,
anatomy and function



D. Measure heartbeat



1/1*

A 58 year old man with .20
history of old anterior myocardial
infarction. Echocardiography
showed severely reduced
systolic function. Which of the
:following is the right answer



A. Ejection fraction= 55%



B. Ejection fraction= 50%



C. Ejection fraction= 40%



D. Ejection fraction= 25%



1/1*

Which drug of the following .21



is not a cause of sinus



1/1* Which drug of the following .21 ✓
is not a cause of sinus
?bradycardia



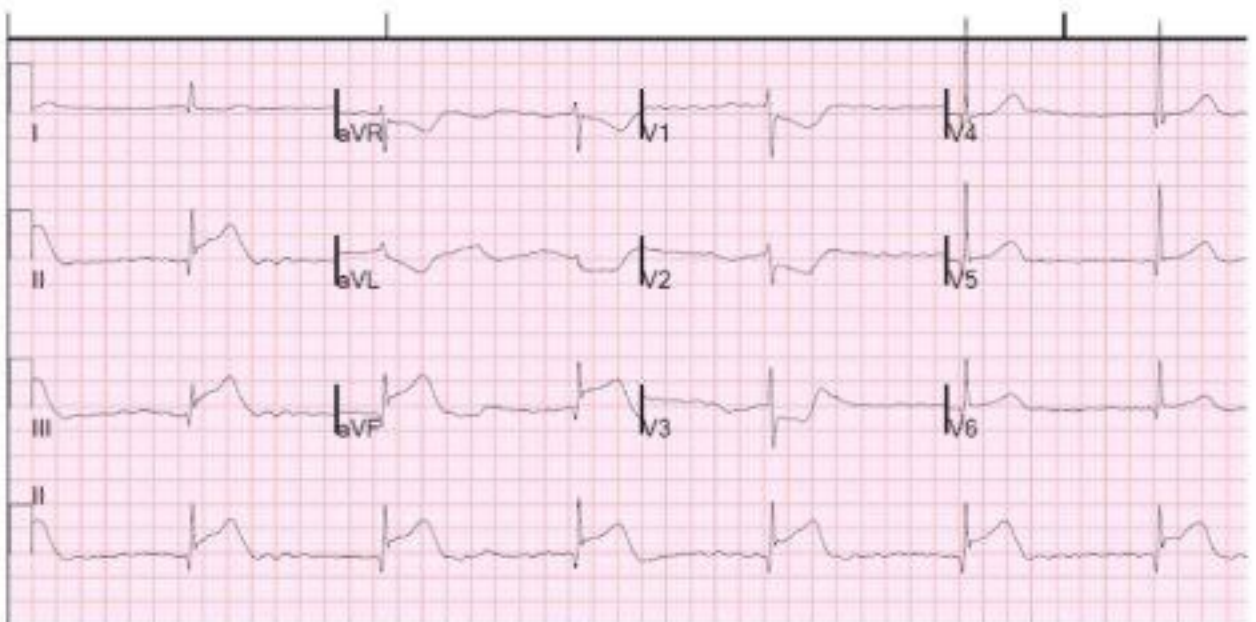
A. Salbutamol ☒

B. Atenolol ☐

C. Verapamil ☐

D. Amiodaron ☐

1/1* What is the etiology of .22 ✓
bradycardia in the following
?ECG



A. Acute anterior myocardial infarction ☐

✓ B. Acute inferior myocardial infarction. ☒

C. Unstable angina. ☐

D. NSTEMI. ☐

1/1* A 2-month-old girl presents .23 ✓

to the emergency department with respiratory distress. Her mother reports that the infant had been well until about 2 weeks ago when she first noticed the infant had rapid breathing. The rapid breathing persists and is present during sleep. The infant had previously fed well but now becomes sweaty and irritable when taking a bottle and takes less than 2 ounces at any one time. The mother is concerned that the infant is losing weight. Physical examination reveals a HR of 140/min, BP 80/45 mmHg, RR 60/min, and temperature 36.8 °C. There is a prominence of the left precordium and a palpable sternal lift. She has a blowing holosystolic murmur that is best heard along the left sternal border. SpO₂ is 95%. Her chest x-ray shows cardiomegaly and increased pulmonary vascular markings. Which of the following is the most likely explanation for this child's symptoms

A. Right-to-left shunt ☐



B. Left-to-right shunt ☒



C. Left ventricular outflow tract obstruction



D. Right ventricular outflow tract obstruction



1/1*

A 63 year male suffering from chest pain upon exertion is diagnosed with classical angina. A drug formulation that you should prescribe for relief of symptoms, taken on need



A. Beta blockers



B. Antiplatelet



C. Sublingual nitrates

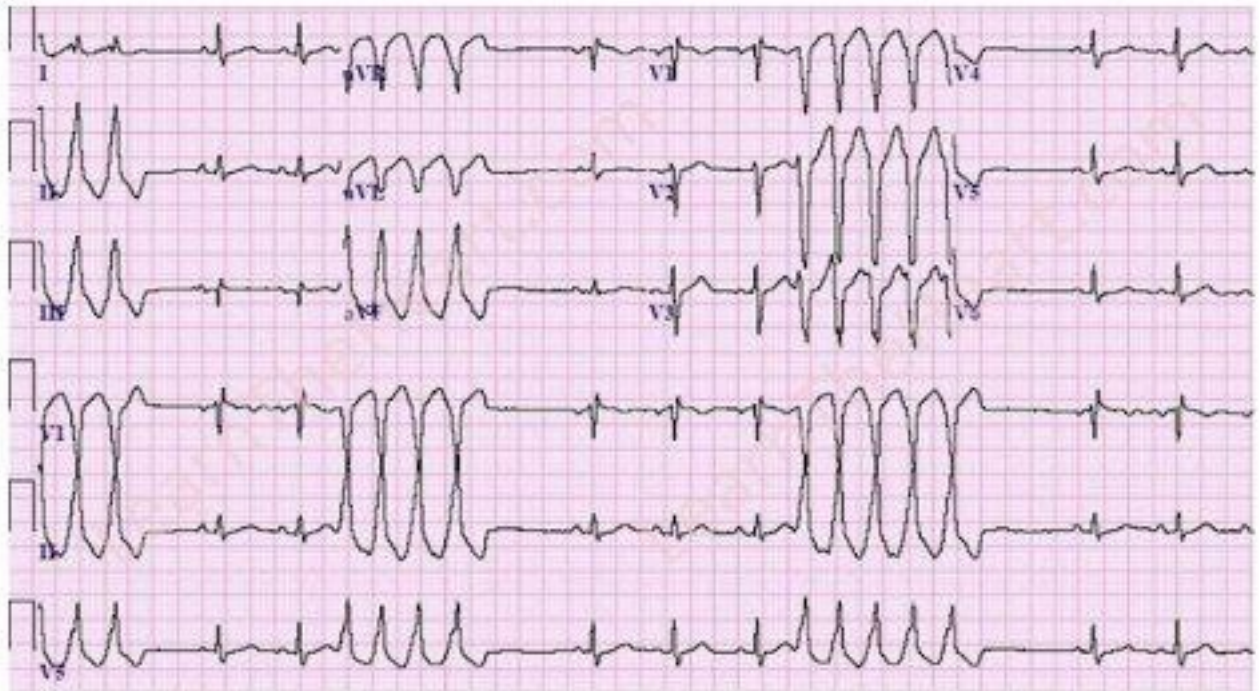


D. Calcium channel blockers.



1/1*

✓ 25. 56 year female patients
complained from dizziness and
palpitation. What is the rhythm in
the following ECG



✓ A. Non sustained ventricular
tachycardia



B. Intermittent right bundle branch
block



C. Artifacts



D. Paroxysmal atrial fibrillation with
aberration



خيار 5



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